

Access guide — pancreatic-mets-kras-g12d-basal-arid1a -k9r3

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	pooled mutant-KRAS peptide vaccine + nivolumab + ipilimumab	vaccine	Off-label use + Clinical trial	investigational vaccine (no approval); the nivolumab and ipilimumab backbone components are FDA-approved in other tumor types	Do not pursue off-label nivolumab + ipilimumab as a standalone strategy here; the MSS / TMB-8.6 profile predicts low benefit and payers will likely deny it
2	ELI-002 (amphiphile mutant-KRAS peptide vaccine, G12D/G12R + Amph-CpG-7909)	vaccine	Clinical trial + Expanded access program	investigational; no approval anywhere	Treat ELI-002 as informational for now; neither the trial nor the EAP fits unresectable metastatic disease
3	INCB161734, oral KRAS G12D-selective inhibitor	small_molecule	Clinical trial	investigational; no approval anywhere	Call the Incyte US call center (1.855.463.3463) or email medinfo@incyte.com to find the nearest DAWN-303 (NCT07522073) site with an open first-line slot
4	anti-KRAS G12D HLA-restricted TCR-engineered T cells (NT-112, AZD0240, and the published C*08:02 product)	CAR-T	Clinical trial + Not yet accessible	investigational; no approval anywhere	Do not pursue NCT06218914; the patient's HLA type (C05:01/C06:02, A01:01/A02:01) does not match either arm
5	ceralasertib (AZD6738), oral ATR inhibitor	small_molecule	Clinical trial	investigational; no approval anywhere	Have the treating team order ARID1A (BAF250a) IHC on tumor tissue; the frameshift call alone does not confirm eligibility

6	daraxonrasib (RMC-6236), pan-RAS(ON) inhibitor	small_molecule	Clinical trial + Expanded access program	investigational; FDA breakthrough therapy designation in PDAC; NDA expected on the strength of RASolute 302 but no approval yet (as of June 2026)	Call the Revolution Medicines study line (1-844-2-REVMED) or email medinfo@revmed.com to find the nearest RASolute 303 (NCT07491445) site with an open first-line slot
7	tuvusertib (M1774), oral ATR inhibitor	small_molecule	Clinical trial	investigational; no approval anywhere	Treat tuvusertib as secondary to ceralasertib on the ATR axis; its ARID1A cohort is endometrial-centered and the trial isn't recruiting
8	zoldonrasib (RMC-9805), KRAS G12D(ON)-selective inhibitor	small_molecule	Clinical trial	investigational; no approval anywhere	Call 1-844-2-REVMED or email medinfo@revmed.com to locate the nearest RASolute 305 (NCT07621718) site with an open first-line slot
9	MRTX1133, KRAS G12D-selective noncovalent inhibitor	small_molecule	Unavailable	investigational; program discontinued	Do not pursue MRTX1133 directly; the program is closed
10	camonsertib (RP-3500), oral ATR inhibitor	small_molecule	Unavailable	investigational; no approval; ATTACC combination trial terminated (no phase 2 expansion). Roche ended its license in 2024; Repare regained rights	Do not pursue camonsertib via ATTACC; it is terminated with no successor combination trial
11	tazemetostat (EPZ-6438), EZH2 inhibitor	small_molecule	Unavailable	previously FDA-approved (epithelioid sarcoma; relapsed/refractory follicular lymphoma); voluntarily withdrawn from all markets by Ipsen on 9 March 2026 over safety signals; all trials and expanded-access programs closed	Do not pursue tazemetostat; it has been withdrawn from all markets and all programs are closed

Off-label use (1)

1. pooled mutant-KRAS peptide vaccine + nivolumab + ipilimumab

Aliases: mKRAS peptide vaccine, pooled mutant KRAS vaccine

Modality: vaccine · **Regulatory:** investigational vaccine (no approval); the nivolumab and ipilimumab backbone components are FDA-approved in other tumor types · **Guidelines:** the vaccine is not in any guideline; nivolumab/ipilimumab are not NCCN-recommended for MSS PDAC and checkpoint monotherapy is low-yield here given MSS, TMB 8.6 · **Geographic scope:** US (single-center academic study, completed) · **Verified:** 2026-06-06

The vaccine itself is a finished single-center academic study with no open slot and no manufacturer to supply it, so the combination as studied is not obtainable. The checkpoint backbone (nivolumab plus ipilimumab) is FDA-approved in other cancers and could in theory be prescribed off-label, but for this MSS, TMB-8.6 tumor that would be low-yield, off-guideline, and very likely denied by payers. The useful read from this row is mechanistic: the immunotherapy route into this tumor runs through KRAS-inhibitor-driven antigen restoration plus checkpoint, not checkpoint or vaccine alone.

Next steps

1. Do not pursue off-label nivolumab + ipilimumab as a standalone strategy here; the MSS / TMB-8.6 profile predicts low benefit and payers will likely deny it
2. Confirm MMR protein IHC (MLH1/PMS2/MSH2/MSH6) on tumor tissue before any checkpoint-based claim, per the case's validation gate
3. Route the checkpoint rationale instead toward KRAS-inhibitor-plus-anti-PD-1 cohorts (e.g. the retifanlimab cohort in INCB161734 NCT06179160)
4. If a research team wants the academic protocol detail, contact nzaidi1@jhmi.edu

Manufacturer / sponsor contact

- **Company:** Johns Hopkins / Sidney Kimmel Comprehensive Cancer Center (academic sponsor of the vaccine); Bristol Myers Squibb markets nivolumab and ipilimumab
- **Med info phone:** 1-800-321-1335
- **Med info email:** medical.communications@bms.com
- **Notes:** The vaccine is an academic construct with no manufacturer to dispense it. The BMS line covers the nivolumab/ipilimumab components only. Academic correspondence: nzaidi1@jhmi.edu (N. Zaidi, Johns Hopkins).

Payer / coverage: The investigational vaccine has no payer pathway. Off-label nivolumab + ipilimumab for MSS PDAC would face prior-authorization denial in most plans, since there is no on-label indication, no NCCN listing, and the MSS / TMB-8.6 profile predicts low benefit from checkpoint blockade. Not a route to pursue on its own.

Notes: Two paths in the array: `clinical_trial_only` reflects the (now completed) academic vaccine study; `off_label_use` reflects the approved nivolumab/ipilimumab components. Neither is a recommended action for this patient given MSS biology.

Clinical trial (7)

2. ELI-002 (amphiphile mutant-KRAS peptide vaccine, G12D/G12R + Amph-CpG-7909)

Aliases: ELI-002, ELI-002 7P, ELI-002 2P, Amph-Peptides, Amph-CpG-7909

Modality: vaccine · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 AMPLIFY-201 showed 84% mKRAS T-cell responses in the MRD/adjuvant setting · **Geographic scope:** US (expanded access protocol is US-only) · **Verified:** 2026-06-06

The KRAS vaccine axis is HLA-agnostic, so this patient's alleles don't rule it out, but the practical problem is setting, not biology: both the ELI-002 trial and its expanded-access protocol are written for the post-resection MRD/adjuvant population, and he has newly diagnosed unresectable metastatic disease. There is no enrollable ELI-002 path for metastatic disease right now. The most a treating team could do is email Elicio to ask whether any metastatic-eligible protocol is planned.

Next steps

1. Treat ELI-002 as informational for now; neither the trial nor the EAP fits unresectable metastatic disease
2. If the team still wants to ask, email clinicaltrialinquiries@elicio.com to check whether any metastatic-setting ELI-002 protocol is open or planned
3. Revisit only if the disease ever becomes resectable / MRD-state, which would change eligibility

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05716264	1	active not recruiting	no	—
NCT07083478	Expanded access	unknown	no	clinicaltrialinquiries@elicio.com

Manufacturer / sponsor contact

- **Company:** Elicio Therapeutics
- **Compassionate / expanded access:** <https://elicio.com/expanded-access-policy/>
- **Compassionate use email:** clinicaltrialinquiries@elicio.com
- **Notes:** Expanded-access requests must come from the treating physician via clinicaltrialinquiries@elicio.com. The EAP is written for the adjuvant indication; a metastatic request would be off-protocol and would need the company's separate agreement.

Payer / coverage: Investigational; no commercial product or payer pathway.

Notes: The load-bearing constraint is the adjuvant/MRD setting, not HLA. Carried so the report documents why the most-discussed KRAS vaccine isn't reachable for a metastatic patient.

3. INCB161734, oral KRAS G12D-selective inhibitor

Aliases: INCB161734

Modality: small_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 monotherapy ORR 37%, DCR 78% in pretreated G12D PDAC (ASCO GI 2026) · **Geographic scope:** US sites (US call center 1.855.463.3463); ex-US sites via +800 00027423 / eumedinfo@incyte.com · **Verified:** 2026-06-06

INCB161734 carries the strongest single-agent response data surfaced in G12D PDAC (37% ORR), and its

front-line trial DAWN-303 (NCT07522073) takes previously untreated metastatic KRAS G12D PDAC, so it's a directly matched first-line option from a second sponsor. The parent phase 1 (NCT06179160) also enrolls at first line and offers combination cohorts. No approval exists; enrollment is the route, with a case-by-case expanded-access ask as a fallback.

Next steps

1. Call the Incyte US call center (1.855.463.3463) or email medinfo@incyte.com to find the nearest DAWN-303 (NCT07522073) site with an open first-line slot
2. Confirm the documented KRAS G12D result, which this trial requires
3. Ask about the parent phase 1 (NCT06179160) combination cohorts if a DAWN-303 slot isn't available
4. If he later becomes trial-ineligible, have the treating physician email globalmedinfo@incyte.com to ask whether expanded access is possible

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07522073		recruiting	likely	Incyte Corporation Call Center (US); medinfo@incyte.com ; 1.855.463.3463
NCT06179160		recruiting	likely	Incyte Corporation Call Center (US); medinfo@incyte.com ; 1.855.463.3463

Manufacturer / sponsor contact

- **Company:** Incyte Corporation
- **Med info phone:** 1.855.463.3463
- **Med info email:** medinfo@incyte.com
- **Product info:** <http://www.incytemi.com/>
- **Compassionate use email:** globalmedinfo@incyte.com
- **Notes:** Incyte will consider expanded access on a case-by-case basis through its Global Medical Affairs group; requests route via globalmedinfo@incyte.com and must come from the treating physician. No INCB161734-specific posted EAP protocol was found as of the verification date.

Payer / coverage: Investigational; no commercial product or payer pathway. Trial drug is sponsor-supplied.

Notes: Second-sponsor G12D-selective agent. Adds front-line breadth and a checkpoint-combination cohort relevant to the MSS immunotherapy question.

4. anti-KRAS G12D HLA-restricted TCR-engineered T cells (NT-112, AZD0240, and the published C*08:02 product)

Aliases: NT-112, AZD0240, HLA-C*08:02 anti-KRAS-G12D TCR-T

Modality: CAR-T · **Regulatory:** investigational; no approval anywhere · **Guidelines:** none · **Geographic scope:** US/EU trial sites, but not accessible to this patient on HLA grounds · **Verified:** 2026-06-06

This is the concrete HLA-screening result the case flagged: the published anti-KRAS-G12D TCR-T works through

HLA-C08:02, and the open AstraZeneca trial (NCT06218914) restricts to C08:02 (NT-112) or A*11:01/11:02 (AZD0240). The patient carries none of those alleles, so neither product can present his mutant epitope and he is not eligible. The KRAS-directed cell-therapy axis is foreclosed by his HLA type rather than by drug availability.

Next steps

1. Do not pursue NCT06218914; the patient's HLA type (C05:01/C06:02, A01:01/A02:01) does not match either arm
2. If any future KRAS G12D TCR-T product restricted to A02:01 or A01:01 opens, re-screen against his actual alleles before considering it
3. Keep the small-molecule RAS(ON) inhibitors as the active KRAS axis instead

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06218914		recruiting	no	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com; 1-877-240-9479

Manufacturer / sponsor contact

- **Company:** AstraZeneca (NCT06218914; NT-112 originated with Neogene/AstraZeneca, AZD0240 is AstraZeneca)
- **Med info phone:** 1-800-236-9933
- **Med info email:** information.center@astrazeneca.com
- **Notes:** AstraZeneca US medical information: 1-800-236-9933. Trial information center: 1-877-240-9479 / information.center@astrazeneca.com.

Payer / coverage: Investigational; no payer pathway. Moot here given the HLA mismatch.

Notes: not_yet_accessible is listed first because the barrier is the patient's HLA genotype, not lack of an open trial; clinical_trial_only reflects that the only route for any patient is enrollment. This documents case judgment call (b).

5. ceralasertib (AZD6738), oral ATR inhibitor

Aliases: ceralasertib, AZD6738

Modality: small_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 2 activity in ARID1A-deficient tumors concentrated in gynecologic histologies, weaker non-gynecologic signal · **Geographic scope:** US (UCSF-sponsored) · **Verified:** 2026-06-06

Ceralasertib is the cleanest live option on the ARID1A synthetic-lethal axis, and the UCSF basket (NCT03682289) explicitly takes pancreatic cancer and assigns by ARID1A/BAF250a status. Two things stand between the patient and a slot: the trial is active-not-recruiting, so a new opening isn't guaranteed, and entry depends on BAF250a (ARID1A) protein loss by IHC rather than the frameshift call alone. The validation step is ordering ARID1A IHC before pursuing this.

Next steps

1. Have the treating team order ARID1A (BAF250a) IHC on tumor tissue; the frameshift call alone does not confirm eligibility
2. Contact the NCT03682289 site (UCSF; PI Rahul Aggarwal) through the cancer center to ask whether the ARID1A pancreatic arm has any remaining slot despite the active-not-recruiting status
3. For drug-level questions, call AstraZeneca medical information at 1-800-236-9933
4. Keep this secondary to the KRAS axis; pursue in parallel only if ARID1A IHC confirms loss

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03682289		active not recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** University of California San Francisco (sponsor) / AstraZeneca (drug supply)
- **Med info phone:** 1-800-236-9933
- **Med info email:** information.center@astrazeneca.com
- **Notes:** Academic-sponsored trial; AstraZeneca supplies ceralasertib. The AstraZeneca US medical-information line (1-800-236-9933) handles drug-level inquiries. No posted ceralasertib compassionate-use program for this setting was found.

Payer / coverage: Investigational; no payer pathway.

Notes: The BAF250a-IHC gate is the load-bearing constraint, alongside the trial's recruitment status. This is the more active of the two ARID1A routes (ATR vs EZH2).

6. daraxonrasib (RMC-6236), pan-RAS(ON) inhibitor

Aliases: daraxonrasib, RMC-6236

Modality: small_molecule · **Regulatory:** investigational; FDA breakthrough therapy designation in PDAC; NDA expected on the strength of RASolute 302 but no approval yet (as of June 2026) · **Guidelines:** not in NCCN as an approved therapy; pivotal RASolute 302 read out at ASCO 2026 (OS 13.2 vs 6.7 mo, HR 0.40), filing pending · **Geographic scope:** US sites; Revolution Medicines runs trials in the US and internationally · **Verified:** 2026-06-06

The realistic path right now is a front-line trial: RASolute 303 (NCT07491445) takes treatment-naive metastatic PDAC and accepts the KRAS G12D under its RAS-mutant arm, with RMC-GI-102 (NCT06445062) as a backup that adds combination cohorts. There's also an FDA-authorized expanded access program, but it only opens after a prior line of metastatic therapy, so it isn't a door for him until he's progressed on first-line chemo. The one thing worth nailing down before screening is a tissue NGS G12D result to back up the ctDNA call.

Next steps

1. Call the Revolution Medicines study line (1-844-2-REVMED) or email medinfo@revmed.com to find the

nearest RASolute 303 (NCT07491445) site with an open first-line slot

2. Ask the treating team to confirm a tissue NGS KRAS G12D result, since the current call is ctDNA-only and tissue confirmation strengthens trial eligibility
3. If no RASolute 303 site is reachable, ask the same line about RMC-GI-102 (NCT06445062) G12D subprotocols
4. Keep the daraxonrasib EAP (NCT07573215) on file as the off-trial route for later, once he has had a line of metastatic chemotherapy

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07491445	1	recruiting	likely	Revolution Medicines Study Director; medinfo@revmed.com; 1-844-2-REVMED
NCT06445062	2	recruiting	likely	Revolution Medicines; medinfo@revmed.com; 1-844-2-REVMED
NCT06625320	3	active not recruiting	no	Revolution Medicines Study Director; medinfo@revmed.com; 1-844-2-REVMED
NCT07573215	Expanded access	unknown	no	Revolution Medicines Study Director; medinfo@revmed.com; 1-844-2-REVMED

Manufacturer / sponsor contact

- **Company:** Revolution Medicines, Inc.
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** medinfo@revmed.com
- **Compassionate use email:** medinfo@revmed.com
- **Notes:** The EAP and trial inquiries route through the same medical-information line. Requests must come from the treating physician; Revolution Medicines cannot take them from patients or families directly.

Payer / coverage: No commercial product yet, so there is no payer coverage to arrange. Trial drug and the EAP drug are sponsor-supplied at no charge. If daraxonrasib is approved later, coverage will follow the label.

Notes: Daraxonrasib is the strongest efficacy anchor on the KRAS axis for this case but is not yet approved anywhere, so access is trial or EAP only. The EAP line-of-therapy gate is the load-bearing constraint for a treatment-naïve patient.

7. tuvusertib (M1774), oral ATR inhibitor

Aliases: tuvusertib, M1774

Modality: small_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO · **Geographic scope:** US / EU sites (EMD Serono / Merck KGaA) · **Verified:** 2026-06-06

Tuvusertib targets the same ATR axis as ceralasertib, but its ARID1A loss-of-function cohort (Part A3 of DDRiver 320) is built around endometrial cancer, and the relevant parts don't name pancreatic cancer. The trial is also active-not-recruiting. So this is mechanism support for the ATR route, not a slot a PDAC patient can take right now. Ceralasertib's NCT03682289 is the ATR trial that actually lists pancreatic eligibility.

Next steps

1. Treat tuvusertib as secondary to ceralasertib on the ATR axis; its ARID1A cohort is endometrial-centered and the trial isn't recruiting
2. If wider solid-tumor eligibility is unclear, have the team check the current protocol or email EMD Serono medical information via medical.emdserono.com
3. Prioritize ARID1A (BAF250a) IHC first, since any ATR-axis entry depends on confirmed protein loss

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05391683	1B	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** EMD Serono Research & Development Institute (Merck KGaA, Darmstadt)
- **Product info:** <https://www.emdserono.com/us-en/expertise/healthcare-professionals.html>
- **Notes:** US medical-information resources at [medical.emdserono.com](https://www.emdserono.com). US patient-safety / adverse-event line: 1-800-283-8088 ext 5563. No posted tuvusertib compassionate-use protocol for ARID1A PDAC was found.

Payer / coverage: Investigational; no payer pathway.

Notes: Carried as ATR-axis mechanism support. Tumor-type breadth is the load-bearing constraint.

8. zoldonrasib (RMC-9805), KRAS G12D(ON)-selective inhibitor

Aliases: zoldonrasib, RMC-9805

Modality: small_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 ORR ~30% in pretreated G12D PDAC (ASCO GI 2025) · **Geographic scope:** US sites; sponsor also runs trials internationally · **Verified:** 2026-06-06

Zoldonrasib is the G12D-selective agent that maps to this patient's exact allele, and the front-line trial RASolute 305 (NCT07621718) takes treatment-naive metastatic KRAS G12D PDAC, so it's the most precisely matched enrollable option on the KRAS axis. RMC-GI-102 (NCT06445062) is the platform-study backup. No approval and no standalone expanded-access program exist, so enrollment is the only route.

Next steps

1. Call 1-844-2-REVMED or email medinfo@revmed.com to locate the nearest RASolute 305 (NCT07621718) site with an open first-line slot
2. Confirm the tissue/ctDNA KRAS G12D result is documented, since this trial requires G12D specifically

3. If RASolute 305 isn't reachable, ask the same line about zoldonrasib cohorts in RMC-GI-102 (NCT06445062)
4. Note that the first-in-human NCT06040541 basket requires prior therapy and won't take a treatment-naive patient yet

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07621718		recruiting	likely	Revolution Medicines Study Director; medinfo@revmed.com; 1-844-2-REVMED
NCT06445062		recruiting	likely	Revolution Medicines; medinfo@revmed.com; 1-844-2-REVMED
NCT06040541		recruiting	no	Revolution Medicines, Inc.; medinfo@RevMed.com; 1-844-2-REVMED

Manufacturer / sponsor contact

- **Company:** Revolution Medicines, Inc.
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** medinfo@revmed.com
- **Notes:** Same sponsor and medical-information line as daraxonrasib. No standalone zoldonrasib EAP was found as of the verification date.

Payer / coverage: Investigational; no commercial product or payer pathway. Trial drug is sponsor-supplied.

Notes: Distinct from pan-RAS daraxonrasib: zoldonrasib is G12D-selective and pairs to the patient's allele directly. The line-of-therapy gate on the FIH basket is the only place he'd currently be excluded.

Unavailable (3)

9. MRTX1133, KRAS G12D-selective noncovalent inhibitor

Aliases: MRTX1133

Modality: small_molecule · **Regulatory:** investigational; program discontinued · **Guidelines:** none · **Geographic scope:** not applicable (discontinued) · **Verified:** 2026-06-06

This one is a dead end. MRTX1133 was the first clinical KRAS G12D-selective small molecule, but the program was terminated for formulation problems before any efficacy read-out, so there is no trial slot, no expanded access, and no approval. The mechanism it validated lives on in zoldonrasib and INCB161734, which are the agents to pursue instead.

Next steps

1. Do not pursue MRTX1133 directly; the program is closed
2. Route the same KRAS G12D-selective rationale to zoldonrasib (RASolute 305, NCT07621718) and INCB161734 (DAWN-303, NCT07522073), which carried the mechanism forward

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT057317206		terminated	no	—

Manufacturer / sponsor contact

- **Company:** Mirati Therapeutics (acquired by Bristol Myers Squibb)
- **Med info phone:** 1-800-321-1335
- **Med info email:** medical.communications@bms.com
- **Notes:** Mirati is now part of Bristol Myers Squibb; the BMS medical-information line is listed for completeness, though there is nothing to dispense. Adverse-event line: 1-800-721-5072 / worldwide.safety@bms.com.

Payer / coverage: No product; nothing to cover.

Notes: Included so the report shows explicitly that this first-generation G12D drug cannot be obtained, rather than leaving the row silently absent.

10. camonsertib (RP-3500), oral ATR inhibitor

Aliases: camonsertib, RP-3500

Modality: small_molecule · **Regulatory:** investigational; no approval; ATTACC combination trial terminated (no phase 2 expansion). Roche ended its license in 2024; Repare regained rights · **Guidelines:** none · **Geographic scope:** not applicable (relevant trial terminated) · **Verified:** 2026-06-06

Camonsertib gave ATR-class precedent, but the trial that surfaced here (ATTACC, NCT04972110) is terminated with no phase 2 expansion, didn't confirm ARID1A in its biomarker list, and didn't establish pancreatic eligibility. There is no open camonsertib slot for this patient and no posted expanded-access route. On the ATR axis, ceralasertib's pancreatic-eligible basket is the option to pursue instead.

Next steps

1. Do not pursue camonsertib via ATTACC; it is terminated with no successor combination trial
2. Use ceralasertib (NCT03682289) as the live ATR-axis route, gated on ARID1A (BAF250a) IHC
3. Only contact Repare directly if a clinician specifically wants to ask about any newer camonsertib protocol, since none was found as of the verification date

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04972110		terminated	no	—

Manufacturer / sponsor contact

- **Company:** Repare Therapeutics
- **Product info:**<https://www.reparerx.com/camonsertib-atr-inhibitor/>
- **Notes:** Repare regained global rights after Roche exited the collaboration in 2024. The ATTACC combination study is terminated; no posted compassionate-use route for ARID1A PDAC was found. Any access ask would have to go directly to Repare.

Payer / coverage: Investigational; no payer pathway.

Notes: unavailable because the surfaced trial is terminated and no ARID1A-selected or PDAC-eligible camonsertib program is currently open. Included so the report shows the ATR axis narrows to ceralasertib.

11. tazemetostat (EPZ-6438), EZH2 inhibitor

Aliases: tazemetostat, EPZ-6438, Tazverik

Modality: small_molecule · **Regulatory:** previously FDA-approved (epithelioid sarcoma; relapsed/refractory follicular lymphoma); voluntarily withdrawn from all markets by Ipsen on 9 March 2026 over secondary-malignancy safety signals; all trials and expanded-access programs closed · **Guidelines:** prior NCCN listings in its approved indications are moot post-withdrawal; never guideline-supported for PDAC · **Geographic scope:** not applicable (withdrawn worldwide) · **Verified:** 2026-06-06

Tazemetostat is off the table. Although it was FDA-approved in epithelioid sarcoma and follicular lymphoma, Ipsen voluntarily withdrew it from all markets in March 2026 over secondary-malignancy safety signals and shut down every trial and expanded-access program, so there is no supply route, no off-label path, and no ARID1A basket left. The EZH2 route into this patient's ARID1A axis has lost its lead agent; the ATR route (ceralasertib) is the one still standing.

Next steps

1. Do not pursue tazemetostat; it has been withdrawn from all markets and all programs are closed
2. Direct the ARID1A synthetic-lethal rationale to the ATR axis (ceralasertib, NCT03682289) instead
3. Note for the board that the EZH2-in-ARID1A hypothesis lost both its lead agent and its supporting efficacy signal (negative NRG-GY014)

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05023655		terminated	no	—

Manufacturer / sponsor contact

- **Company:** Ipsen Biopharmaceuticals (US marketer of Tazverik)
- **Notes:** Ipsen withdrew tazemetostat in all indications from all markets on 9 March 2026 and discontinued all active trials and expanded-access programs. There is no current supply route. The withdrawal was driven by secondary hematologic malignancy signals in the SYMPHONY-1 trial, not by an ARID1A-specific finding.

Payer / coverage: No product on the market; nothing to cover. Off-label use is not possible because the drug has been withdrawn from supply entirely.

Notes: Status is unavailable rather than off_label_use: even though the drug carried FDA approval, the March 2026 worldwide withdrawal removes any supply, so off-label prescription is not actually possible. Included so the report shows explicitly that the EZH2 route is closed.
